Total Health

From population-average guidelines
to patient-specific truth.

Four threads, one thesis

SECURED

$6M

Pre-seed closed — $5M from Vinod Khosla, $1M from aligned angels. Enough to prove the wedge, not enough to skip proving it.

SECURED

Core wedge chosen

Hereditary cancer & genomic risk is our resourced execution focus. Rare disease is a smaller, deliberate science bet running alongside it.

ACTIVE

~25

An early computational signal from our work on SLC6A1, a rare pediatric epilepsy gene — candidates who may be naturally protected, or may simply be unaffected carriers. Validation is next.

ACTIVE

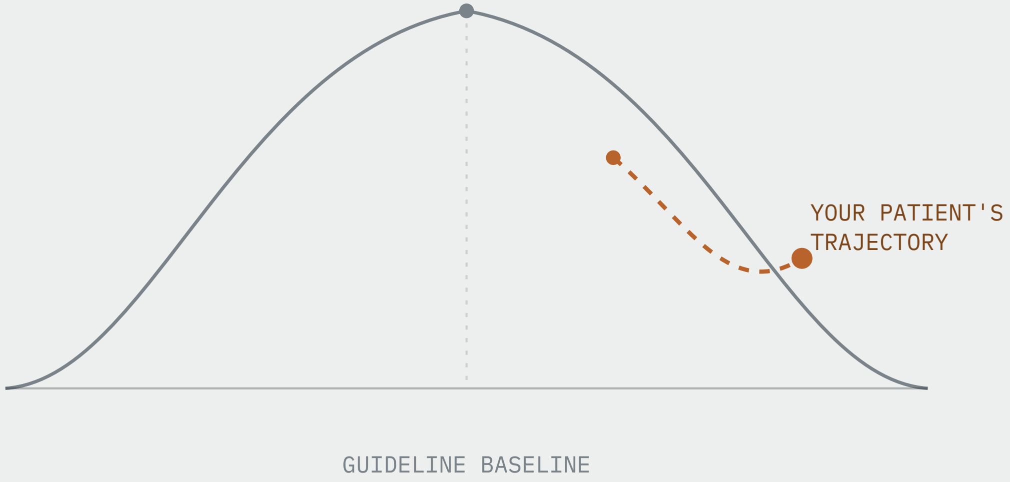
3

Active conversations with Pathos, CZI, and Pfizer. Mount Sinai is next — we're building patient-volume proof before that first pitch.

Medicine treats the average patient. We treat yours.

Modern medicine is built for population averages — guidelines, snapshots, specialty silos. That isn't a failure of physicians. It's a failure of system design: no clinician has the tooling to integrate a patient's full trajectory against the entire evidence base, for every patient, every time.

Total Health is building the layer that does this: a physician-supervised Clinical AI Reasoner that knows the standard of care cold, and knows precisely when a patient's biology justifies doing something different. Not a chatbot. Not a guideline summarizer — a reasoning system a physician can trust, inspect, and act on.



Guidelines describe the median patient. Our reasoner knows when yours is not the median — and can show its work explaining why.

This isn't a failure of physicians. It's a failure of system design.

TODAY	WITH TOTAL HEALTH
A one-time snapshot assessment	A continuously updated model of the patient's trajectory
One-size-fits-all guidelines	Guidelines as the starting point, plus reasoning about this patient's specific biology
Fragmented specialist opinions	Cross-specialty synthesis, one coherent view
A static report or note	A recommendation you can see the reasoning behind
Learning arrives on a publication cycle	Learning happens continuously, from real outcomes
Eligibility decided by a single biomarker label	Prediction based on what's actually driving this patient's disease — not just one test result

> *The highest-value biomedical decisions come from identifying the mechanism actually driving a specific patient's biology, connecting it to real outcomes — and only then choosing an intervention.*

That lesson was learned building the reasoning models behind Pathos's Oncology Foundation Model — work Eric Schadt led before joining Total Health full time. Vinod Khosla backed this exact thesis with a $5M pre-seed check.

Ambient-scribe copilots capture the visit. Guideline-retrieval tools summarize the literature. Neither builds a mechanism-linked model trained on real outcomes, inside an actual care relationship — that takes being the provider, which only a provider-first company can do.

We sit inside the patient relationship, not outside it — so we control the data, the workflow, and the incentives needed to make this compound over time.

TARGET ARCHITECTURE — PROTOTYPE IN BUILD

Every recommendation comes with its receipts

1. **Patient state**
   Full timeline: labs, imaging, genomics, treatment history, family history.
2. **Standard-of-care baseline**
   What the guidelines say, and why — the floor, not the ceiling.
3. **What this patient's biology says**
   Checked against real-world outcomes, not just this patient's chart.
4. **Recommendation package**
   A clear recommendation, alternatives considered, uncertainty flagged.
5. **Physician review**
   A physician accepts, edits, or rejects. The outcome feeds back in.

> *The physician is never replaced. The system is never a black box — every recommendation shows the standard path, the patient-specific reason to consider another, and how confident we are.*

Real patients, starting now

Our first wedge: identify patients eligible for hereditary cancer & genomic risk testing, guide them through testing and counseling, and connect positive results to the right follow-up care — sequenced through our whole-genome sequencing partner, the New York Genome Center (NYGC), today a manual, high-touch workflow we're actively productizing.

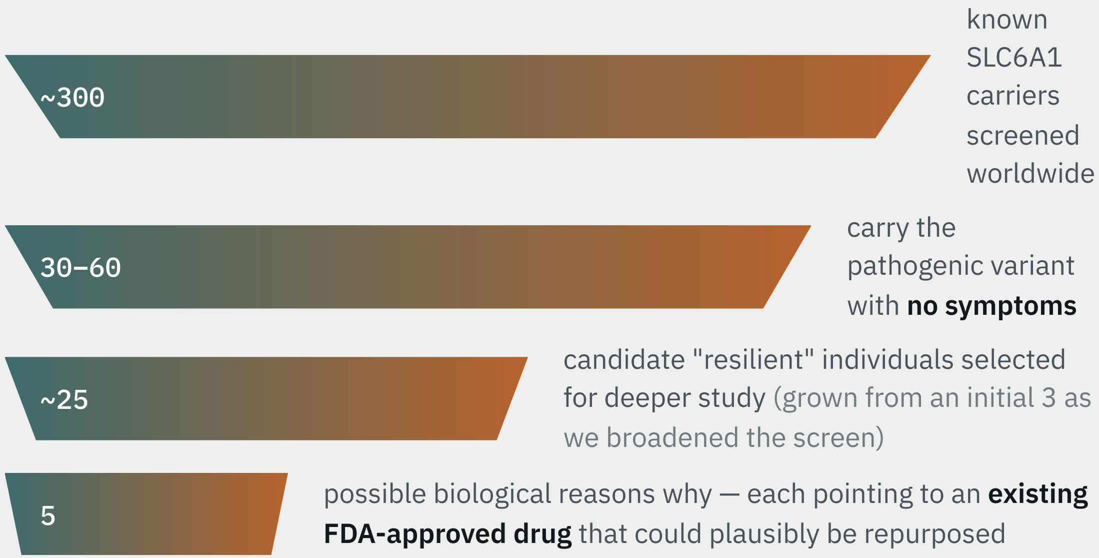
1. **Signup** — Patient identified & enrolled.
2. **Testing** — Genetic test ordered & sequenced.
3. **Counseling** — Pre- and post-test counseling.
4. **Consult** — Provider consult on findings.
5. **Care plan** — Personalized follow-up plan.

BUILDING NOW

NEXT — 25–50 PATIENT PILOT WITHIN ~2 MONTHS OF FIRST CASES

THEN — 100 PATIENTS HERE + 10 IN PHARMACOGENOMICS/RARE DISEASE/CARDIOMYOPATHY, BEFORE WE BRING MOUNT SINAI A FULL INTEGRATION PITCH

What if nature already solved this?



Some of the most important drug targets in history came from finding people who should be sick — and aren't. CCR5 deletion carriers, naturally resistant to HIV, led directly to a new class of therapy. We're applying the same logic to rare genetic disease: find people who carry the broken gene but never got sick, and figure out what's protecting them.

Our flagship case is **SLC6A1**, a rare pediatric epilepsy-linked gene — chosen because it started close to home for our team.

● GRANT SUBMITTED TO CZI

Decision under review — originally expected in June

> *The open question we haven't answered yet: are these 25 people protected by biology — or simply unaffected carriers, a common, unremarkable pattern called incomplete penetrance? Functional validation is what tells the two apart.*

Building the relationships the reasoner needs

PREP STAGE

Mount Sinai

A potential first major health-system partner — starting with hereditary cancer genomics and rare-disease/pediatric care, and potentially widening into our full reasoning platform. We haven't made the formal approach yet: we want real patient volume across both clinical flows first, not just a pitch.

ACTIVE

Pathos

In active discussions around access to their oncology foundation model and related tumor RNA/outcomes data, to apply to patient-specific precision oncology — with Total Health as the clinical delivery, orchestration, and reasoning layer. We see Pathos as one component of a broader platform, not a dependency.

ACTIVE

CZI

A multi-year grant application is under review to fund our rare-disease / resilience program at scale, alongside patient-advocacy and sequencing partners including SLC6A1 Connect, Combined Brain, and NYGC.

ACTIVE

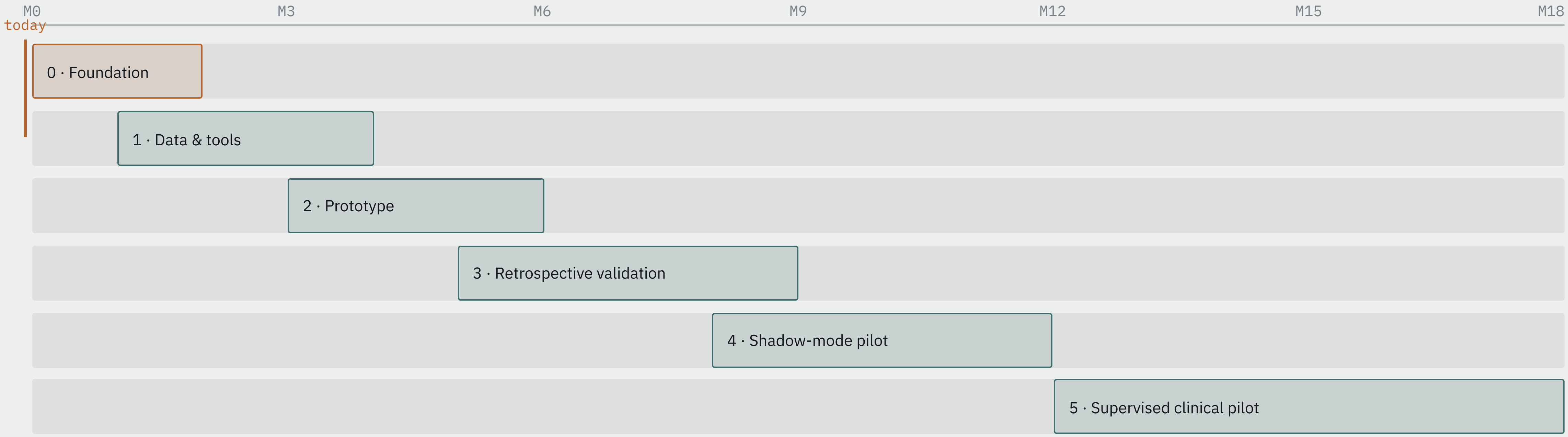
Pfizer

Early conversations about a strategic AI partnership, applying our approach to Pfizer's clinical trial and oncology data — a non-dilutive way to get data and compute access, without touching our rights over patient care.

To be clear: none of these are signed yet. This is our live pipeline, not a list of done deals.

From first prototype to real patients

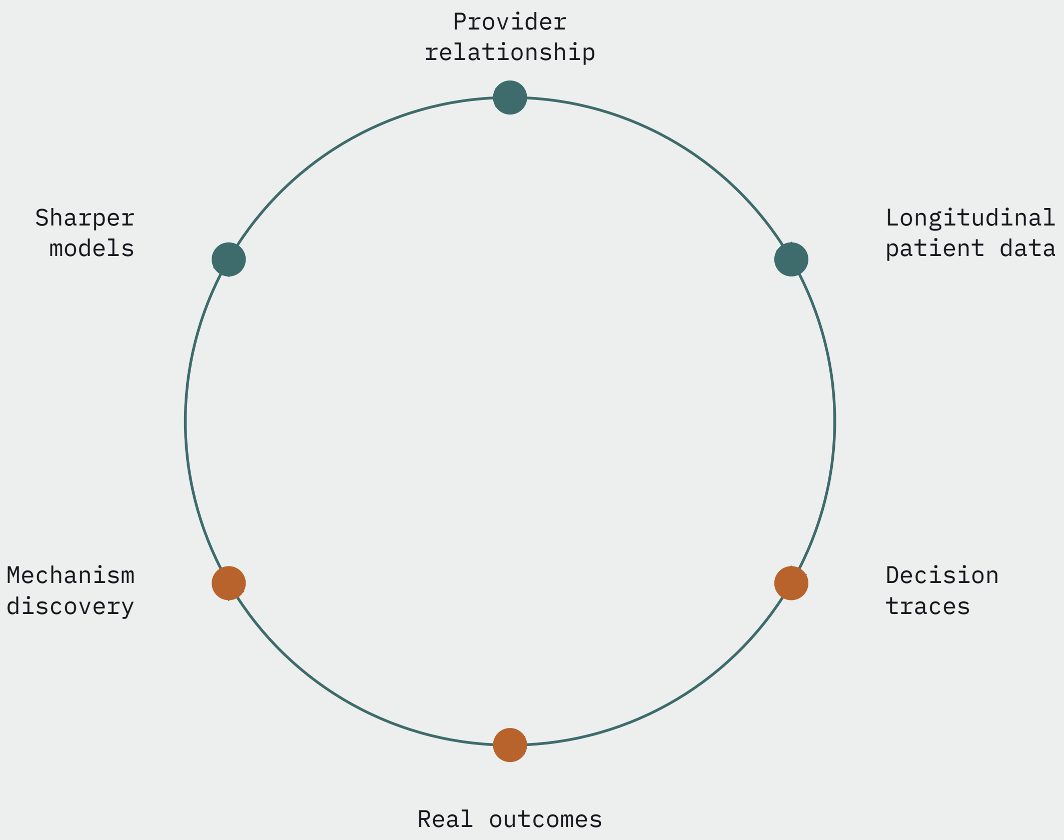
This roadmap runs on the $6M we've already raised — it doesn't assume Mount Sinai, Pathos, CZI, or Pfizer closes. Hereditary cancer & genomic risk is our core, fully-resourced wedge; the rest is upside on top of it, not a dependency.



The loop is the moat

Models alone don't stay a moat — they commoditize. Ours compounds because of the loop: provider relationship, patient data, decision traces, real outcomes, discovered mechanisms, sharper models — and back again.

Health systems and pharma partners already sit on enormous data — but none are built to run this loop themselves end to end. That's nearly impossible to copy from outside a real care relationship, and it's the gap each of our conversations is trying to fill with us, not around us.



What would change our mind

- **No signed partnership yet.** Mount Sinai, Pathos, CZI, and Pfizer are all live conversations, not commitments. Data-access economics with model/data partners like Pathos are unresolved and may not close on the terms we want.
- **CZI's funding decision is still outstanding** on our rare-disease grant application, past our original expected timing — we're continuing the program either way.
- **SLC6A1 mechanisms are computational leads,** not clinical facts. Our strongest candidates could reflect real protection, or simply incomplete penetrance — a common, unremarkable genetic pattern. Functional validation will tell us which.
- **Delivering care alongside a health system** requires a corporate structure (an MSO/PC model) we're still finalizing, alongside the broader regulatory posture — decision support vs. device — we're defining with counsel.
- **The roadmap is ambitious for our stage,** on purpose — we're sequencing around one core wedge, not the whole vision, so we can prove it before we scale it.

All in

Starting this summer, **Eric Schadt** is full time on Total Health — wrapping up his Chief AI Officer chapter at Pathos to do it. He led the reasoning-model work behind Pathos's Oncology Foundation Model, the same insight now driving our Clinical AI Reasoner.

Hiring priorities

- Clinical & data engineering core
- Mechanism-modeling scientists
- Physician-facing product
- Evaluation & safety

Where you come in

Health-system introductions

To clinical & AI leadership at systems beyond Mount Sinai.

Pharma introductions

To partners serious about building AI into how they discover drugs, alongside Pfizer.

Talent referrals

Clinical, mechanism-modeling, and evaluation/safety talent.

Patience

To run the proof points that de-risk the next chapter, properly.

One thing we're deliberately not asking for here: capital. This is a progress update, not a raise — we'll bring a clear read on runway and next-raise timing when that conversation is next.

We're early.
That's the point.

The proof points ahead — patient volumes, validated mechanisms, signed partnerships — are what convert a strong thesis into a durable company. Grateful to be building this with you.

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